

August 12, 2026

Ambetter from Absolute Total Care
Provider Appeals Department

RE: Formal Appeal of Claim Denial — Keitha Turner (MRN MUSC0626052), Claim MUSC-20260315-6CD1-1

Patient	Keitha Turner (DOB 2004-07-22)
MRN	MUSC0626052
Member ID / Group	AMB999437436 / GRP-60820
Claim number	MUSC-20260315-6CD1-1
Date of service	2026-03-15
Procedure billed	CPT 99214 — Office/outpatient visit, established patient, moderate complexity
Primary diagnosis	J06.9 — Acute upper respiratory infection, unspecified
Denied amount	\$590.37
Denial code	CARC 197 / RARC N54 — Precertification/authorization/notification/pre-treatment absent.
Appeal deadline	2026-07-08

Dear Appeals Review Committee:

SUMMARY OF APPEAL

MUSC Health is submitting this formal appeal on behalf of Keitha Turner (Member ID: AMB999437436) regarding the denial of Claim MUSC-20260315-6CD1-1 for services rendered on March 15, 2026, at MUSC Health East Cooper Medical Center. The claim was denied under CARC 197 ("Precertification/authorization/notification/pre-treatment absent") and RARC N54 ("Claim information is inconsistent with pre-certified/authorized services"), with the payer remark stating: "No prior authorization on file for the date of service billed." MUSC Health respectfully contests this denial and requests full reconsideration and payment of the allowed amount of \$321.00.

CLINICAL BACKGROUND

Ms. Turner is a 22-year-old established patient who presented on March 15, 2026, for an outpatient office visit with Dr. Elena V. Castellanos, MD (NPI 1881726354) at MUSC Health East Cooper Medical Center. The visit was coded as CPT 99214, an office or outpatient visit for an established patient of moderate complexity, with a primary diagnosis of J06.9 (Acute upper respiratory infection, unspecified). The service was rendered in a campus outpatient hospital setting (Place of Service 22, Revenue Code 0510). Ms. Turner has a documented clinical history relevant to her ongoing care, including prior episodes of acute viral pharyngitis, a history of subcutaneous immunotherapy, allergy screening, and active prescriptions including an epinephrine auto-injector and fexofenadine, indicating a background of allergic disease that contextualizes the complexity of her evaluation.

BASIS FOR APPEAL

The denial of this claim on prior authorization grounds is not consistent with standard payer policy as it applies to evaluation and management services for acute, unscheduled conditions. CPT 99214 represents a medically necessary office visit for an established patient presenting with an acute upper respiratory infection. Acute, episodic outpatient evaluation and management visits of this nature are broadly recognized across the industry as services that do not require prior authorization, as they are neither elective procedures nor scheduled interventions that would afford the treating provider an opportunity to obtain precertification in advance. Requiring prior authorization for an unscheduled acute illness visit would be inconsistent with timely access-to-care obligations under applicable Marketplace plan requirements and standard managed care practice.

Furthermore, RARC N54 indicates that the claim information is inconsistent with pre-certified or authorized services. To the extent that no authorization was sought, this is because no authorization requirement applies to this category of service. The absence of an authorization on file does not constitute a clinical or administrative deficiency on the part of MUSC Health or the rendering provider; rather, it reflects the nature of the service itself. Dr. Castellanos appropriately documented and billed the encounter in accordance with CPT and ICD-10 guidelines. The billed charge of \$590.37 and the payer's own allowed amount of \$321.00 confirm that the service was recognized as a covered benefit — the sole basis for non-payment is the authorization issue, which MUSC Health submits is inapplicable here.

REQUESTED ACTION

MUSC Health respectfully requests that Ambetter from Absolute Total Care reverse the denial of Claim MUSC-20260315-6CD1-1 and issue payment of the allowed amount of \$321.00 for CPT 99214 rendered on March 15, 2026. Should the plan maintain that a prior authorization requirement applies to this service, we further request that the plan provide, in writing, the specific benefit provision or clinical policy that mandates precertification for unscheduled acute evaluation and management visits billed under CPT 99214 in an outpatient hospital setting. This appeal is submitted within the appeal deadline of July 8, 2026, through the secure provider portal as directed.

Respectfully submitted,

Angela R. Whitfield, RN, BSN, CPC

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Enclosures

- Remittance advice / explanation of payment
- Itemized claim detail (UB-04 / CMS-1500)
- Relevant clinical documentation from the medical record